

**Laboratory Investigation Report**

Patient Name	: Kumar Akash Sharma	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 27 Y 0 M 7 D /M	OP/IP No	: OP/97848
Max ID/Mobile	: 433629/8752005200	Collection Date/Time	: 18/Aug/2021 02:58PM
Lab ID	: 1266082100431	Receiving Date	: 18/Aug/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 18/Aug/2021

Clinical Biochemistry**Blood Sugar (Random), Fluoride Plasma***

Date	18/Aug/2021 02:58PM	Unit	Bio Ref Interval
Random Glucose UV-Hexokinase	274.95	mg/dl	74 - 140

Interpretation A random plasma glucose of > 200 mg/dl with symptoms of diabetes is diagnostic. A confirmatory fasting plasma Glucose (FPG) test or OGTT should be completed on a different day if the clinical condition of the patient permits.

Kindly correlate with clinical findings

*** End Of Report ***


Dr. Mrinalini Priyadarshini
DCP, DNB (Path.)
Consultant Pathologist



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SIN No: B2B367408, Test Performed at : 896 - Motherland Hospital, Motherland hospital Sec-119

Booking Centre : 1534 - Arctern Healthcare Noida, 2nd Floor, Powerstation by Oyo Pioneer Square, Sector 62, Gurugram, Haryana 122002, 8752
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The : **Max Super Specialty Hospital, Saket (West Block)**
1, Press Enclave Road, Saket, New Delhi - 110 017, Phone: +91-11-6611 5050.

CIN No.: L72200MH2001PLC322854

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Hematology

APTT (Partial Thromboplastin Time), Citrate Plasma*, Sodium Citrate

Date	18/Aug/2021 02:56PM	Unit	Bio Ref Interval
APTT	23.9	Sec	24.9 - 33.3
Photo-Optical-Nephelometry			
Control	28.0	sec	

Interpretation

(Activated Partial Thromboplastin Time; PTT)

APTT is the test which checks the "intrinsic coagulation" pathway and is useful for detecting screening of haemophilia A & B, screening of coagulation inhibitors like lupus anticoagulant.

APTT can also be used to monitor heparin monitoring.

Raised APTT is found in - Liver disease, DIC, heparin treatment, circulating inhibitors (Lupus Anticoagulant), and haemophilia (deficiency of Factor VIII & IX).

Low APTT may be seen in - high concentration Factor VIII and is independent risk factor for increased incidence of thrombus formation.

Advice: - 'APTT mixing study', 'Lupus Anticoagulant test', 'specific Factor(s) assay' may be added on for further evaluation.

Prothrombin Time (with INR), Citrate Plasma*

Date	18/Aug/2021 02:56PM	Unit	Bio Ref Interval
Prothrombin Time (PT)	12.3	Sec	12.1 - 15.1
Photo-Optical-Nephelometry			
MNPT Value	11.4	Sec	
INR	1.08		

Interpretation

(Syn: - Prothrombin Time)

PT is the test which checks the "extrinsic coagulation" pathway and is useful for detecting coagulation deficiency, liver disease and disseminated intravascular Coagulation (DIC).

PT can also be expressed as International normalized ratio (INR) used for monitoring warfarin therapy.

Raised PT value seen in - factor deficiency (Fibrinogen (I), Prothrombin (II), factor V, VII, X), oral anticoagulation therapy, liver diseases, Vitamin K deficiency and DIC.

Advice: - 'PT mixing study', 'specific factor(s) assay' may be added on for further evaluation.



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Hematology

Kindly correlate with clinical findings

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Serology Viral Markers (Rapid)

Test Name	Result	Unit	Bio Ref Interval
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Rapid Card Test - HCV*, Serum

Immunochromatography

HCV Card Test	Negative
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Comment Interpretation

It is only a screening test. All reactive samples should be confirmed by HCV quantitative PCR
A non reactive result does not exclude the possibility of exposure to or infection with HCV.
Patients with auto immune liver disease may show falsely reactive results

Rapid Card Test - HIV I & II*, Serum

Immunochromatography

Rapid Card HIV I & II	Negative
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Comment This is only a screening test. All samples detected reactive must be confirmed by using HIV RNA PCR / three different methods.
A non- reactive test result does not exclude the possibility of exposure to an infection with HIV (Window Period) or low viral load.

Advice:

1. HIV by 3 different methods
2. Confirmatory HIV RNA Quantitative PCR.



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Serology Viral Markers (Rapid)

Test Name	Result	Unit	Bio Ref Interval
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VDRL/RPR*, Serum

RPR(Syphilis) Slide Flocculation	Non Reactive
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Comment

Interpretation

1. It is a screening test for syphilis which is useful for following the progression of disease and response to therapy. Rising titers are of immense value in confirming the diagnosis.
2. Biological false positive reactions exhibit low titers and are seen in conditions like Viral fevers, Mycoplasma infection, Chlamydia infection, Malaria, Immunizations, Pregnancy, Autoimmune disorders & past history of Treponemal infection.
3. It is advisable to confirm the diagnosis by tests such as TPHA & FTA-ABS.
4. A definitive clinical diagnosis should not be made by result of a single test only, but should be made by taking clinical history and other laboratory findings in to account.



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Serology Viral Markers (Rapid)

Test Name	Result	Unit	Bio Ref Interval
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Rapid Card Test - Hepatitis B Surface Antigen*, Serum

Immunochromatography

Rapid Card HBsAg Negative

Comment

Interpretation

This is only a screening test.

All reactive samples should be confirmed by 'HBsAg confirmatory test or HBV DNA PCR'.

A non – reactive does not exclude the possibility of exposure to infection with HBV (window period)

False positive results can be obtained due to the presence of other antigens or elevated levels of RF factor.

Advise: Confirmatory test 'HBsAg Confirmatory Quantitative' test followed by 'HBV DNA quantitative PCR'

Kindly correlate with clinical findings

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